Benchmark- Sherwood
Sherwood, OR
971-930-4777

ErgoScience FCE: *The Physical Work Performance Evaluation™*

Name	*Viktoriya Pulliam*
Evaluation Purpose	*General FCE: Any Job*
Evaluation Date	*11/26/2024*
Evaluator	*Luke Hammer PT, DPT, OCS*

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This report summarizes the results of the ErgoScience FCE: **The Physical Work Performance Evaluation.** This evaluation is substantiated by published peer-reviewed reliability and validity research. Research citations and definitions for the US Department of Labor terms are provided at the end of the report.

SUMMARY OF PERFORMANCE

	Tasks	Client Performance
Weight Handling	Below Waist Lift - floor to waist	30 lb Occas.
	Above Waist Lift - waist to eye	20 lb Occas.
	Carry - two hands	35 lb Occas.
	Carry - one hand	20 lb Occas.
	Pushing	23 lb Occas.
	Pulling	20 lb Occas.
Position Tolerance	Sitting	Frequently
	Standing	Frequently
	Work Arms Overhead - Stand	Never
	Work Bent Over - Stooping	Occasionally
	Kneeling	Occasionally
	Work Bent Over - Sitting	Never
Mobility	Climbing Stairs	Frequently
	Repetitive Squatting	Occasionally
	Walking	Frequently
	Ladder Climbing	Constantly
	Repetitive Trunk Rotation - Sitting	Frequently
	Repetitive Trunk Rotation - Standing	Frequently

SUMMARY OF CLIENT EFFORT

Client demonstrated self-limiting performance, functional inconsistencies or low effort on 2/37 (5.41%) of our consistency measures as follows:

1. **Self-Limiting Performance** on 0/20 FCE tasks is within normal limits.
2. **Functional inconsistencies** noted on 0/14 of functional testing consistency parameters
3. **Low Effort** on 2/3 grip strength consistency measures

OVERALL LEVEL OF WORK

Recommended/safe level of work = Light

- The **Weight Handling** section of the test indicates **Light** level of work.
- The **Position Tolerance** section of the test indicates **Light** level of work.
- The **Mobility** section of the test indicates **Medium** level of work

WORK TOLERANCE

- **Based on the individual task tolerances, Viktoriya Pulliam is CAPABLE of sustaining Light level of work (or work up to the limits described in this report) on a full-time basis. This would indicated she is able to complete an office job with either a light or sedentary overall level of work. She would be unable to work as a housekeeper that she was capable of doing prior to her injuries due to limitations with weight handling and position tolerance.**

SUMMARY OF MEDICAL HISTORY

Primary Diagnosis	*Cervical spine/lumbar spine pain*
Injury/Onset Date	*11/21/2019 and 2/11/2020*
Body Parts Injured	*Low Back, Neck, Elbow (Left), Lower Leg (Both)*
Height/Weight	*5'5", 158 lb*
Starting BP/HR	*122/80, 71 bpm*
Starting Pain	*null/10*
Primary Pain Areas	*Low Back, Neck, Thigh (Both), Lower Leg (Both)*

MEDICAL HISTORY

- Patient reports she was in an MVA Nov 2019 and Feb 2020. Has numbness, tingling and spasms of L>R UE, neck stiffness, elbow pain, aching and twitching in the legs. Symptoms have been worsening over the last couple of years.

MUSCULOSKELETAL SCREEN

- See table on last page

TEST LENGTH & REST BREAKS

- No additional rest breaks were taken

EXIT INTERVIEW

Change in Musculoskeletal Measures from beginning to end:

- Client reported an increase in overall pain to 4.00/10
- Gait pattern leaving the evaluation is same compared to gait pattern used upon arriving for test

FCE TASK DETAILS

Weight Handling

Lift - Floor to Waist - Appropriate

- Completed 30 lbs safely (Medium)
- Pain Score: 4.00
- Pain Location: Low back, elbow
- Ending HR: 94
- Signs of Effort:
  - Hands Slip/Difficulty Holding Box
  - Decreased Box Control
  - Irregular Steps

Lift - Waist Height to Eye Level - Appropriate

- Completed 20 lbs safely (Light)
- Pain Score: 4.00
- Pain Location: Neck pain and low back
- Ending HR: 90
- Signs of Effort:
  - Decreased Box Control
  - Shaking/Quivering

Bilateral Carry - Appropriate

- Completed 35 lbs safely (Medium)
- Pain Score: 4.00
- Pain Location: Low back and neck
- Ending HR: 94
- Signs of Effort:
  - Decreased Box Control
  - Shaking/Quivering
  - Shorter Steps

Unilateral Carry - Appropriate

- Completed 20 lb safely (Light)
- Pain Score: 5.00
- Pain Location: Low back and L elbow
- Ending HR: 93
- Signs of Effort:
  - Hands Slip/Difficulty Holding Box
  - Shaking/Quivering
  - Shorter Steps
  - Decreased Box Control

Dynamic Pushing - Appropriate

- Completed 23 lbs (Medium)
- Pain Score: 4.00
- Pain Location: Elbow and forearm
- Ending HR: 90
- Signs of Effort:
  - Other - Measured with force gage

Dynamic Pulling - Appropriate

- Completed 20 lbs (Light)
- Pain Score: 2.00
- Pain Location: Elbow and low back
- Ending HR: 90
- Signs of Effort:
  - Other - Measured with force gage

Mobility

Climbing Stairs - Appropriate

- Completed 50 of 50 - 100% of task
- Pain Score: 4.00
- Pain Location: Low back
- Ending HR: 115
- Within Normal Limits

Repetitive Squatting - Appropriate

- Completed 25 of 25 - 100% of task
- Pain Score: 7.00
- Pain Location: Low back and neck
- Ending HR: 96
- Moderate Deviations
  - Other - Decreased squat depth with more reps

Walking - Appropriate

- Completed 500 of 500 - 100% of task
- Pain Score: 4.00
- Pain Location: Low back
- Ending HR: 91
- Within Normal Limits

Climbing a Ladder - Appropriate

- Completed 39 of 39 - 100% of task
- Pain Score: 3.00
- Pain Location: Low back
- Ending HR: 108
- Within Normal Limits

Repetitive Trunk Rotation - Sitting - Appropriate

- Completed 25 of 25 - 100% of task
- Pain Score: 4.00
- Pain Location: Neck and low back
- Ending HR: 87
- Minimal Deviations
  - Decreased Trunk Rotation

Repetitive Trunk Rotation - Standing - Appropriate

- Completed 25 of 25 - 100% of task
- Pain Score: 3.00
- Pain Location: Low back
- Ending HR: 88
- Minimal Deviations
  - Decreased Trunk Rotation

Position Tolerance

Sitting - Appropriate

- Completed 05:00 of 5:00 minutes - 100% of task
- Pain Score: 2.00
- Pain Location: Low back
- Ending HR: 76
- Minimal Deviations
  - Decreased Weight Bearing on Buttock
  - Lateral Trunk Shift

Standing - Appropriate

- Completed 05:00 of 5:00 minutes - 100% of task
- Pain Score: 4.00
- Pain Location: Low back
- Ending HR: 75
- Moderate Deviations
  - Decreased Weight Bearing on One Leg
  - Lateral Trunk Shift
  - Adjusting position due to pain

Work Arms Overhead in Standing - Appropriate

- Completed 02:01 of 5:00 minutes - 40.33% of task
- Pain Score: 5.00
- Pain Location: Low back, elbow, and neck
- Ending HR: 77
- Severe Deviations
    - Decreased Weight Bearing on One Leg
    - Lateral Trunk Shift
    - Decreased Neck Extension

Work Bent Over/Stooping - Appropriate

- Completed 04:30 of 5:00 minutes - 90% of task
- Pain Score: 7.00
- Ending HR: 76
- Severe Deviations
    - Decreased Weight Bearing on One Leg
    - Adjusting position due to pain

Kneeling - Appropriate

- Completed 05:00 of 5:00 minutes - 100% of task
- Pain Score: 6.00
- Pain Location: Low back
- Ending HR: 74
- Moderate Deviations

Work Bent Over Sitting - Appropriate

- Completed 02:12 of 5:00 minutes - 44% of task
- Pain Score: 6.00
- Pain Location: Low back
- Ending HR: 75
- Moderate Deviations
    - Supporting Body Weight through Legs
    - Adjusting position due to pain

SELF LIMITING BEHAVIOR

The client did not self-limit on any tests. Self-limiting behavior means that the client stopped the task before a maximum effort was reached. Possible causes of self-limiting behavior include: pain; fear of reinjury or motivation. ErgoScience research indicates that motivated patients self-limit on no more than 20% of test items. If the self-limiting exceeds 20%, then psychosocial and/or motivational factors may be affecting test results.

- **Self Limiting < 20% of tasks = Within normal limits[1]**
- Self-Limiting 21% to 33% of tasks = Exceeds normal limits[1]
- Self-Limiting > 33% of tasks = Significantly exceeds normal limits[1]

[1]When compared to a motivated group of patients who participated in research.

REASONS FOR BEHAVIOR

STATEMENTS ON BEHAVIOR

CONSISTENCY OF EFFORT GRIP TESTING DETAILS

The ErgoScience FCE utilizes a formal consistency of effort protocol established and validated by Stokes et al.[2] In this protocol, three (3) different statistical calculations on grip strength testing data are performed.

STATISTICAL TEST #1

Right Hand

The standard deviation of sustained maximum right grip strength across 5 handle positions was 6.02, indicating a low effort.

Left Hand

The standard deviation of sustained maximum left grip strength across 5 handle positions was 4.03, indicating a low effort.

STATISTICAL TEST #2

The Rapid Exchange Grip (REG) is 2 pounds different from the peak slow sustained grip. Clinical studies demonstrate that this difference is not significant and indicates that the client exerted maximum effort.

STATISTICAL TEST #3

A regression analysis was calculated based on the peak effort of sustained grip and the maximum REG. This calculation indicates that the client gave a low effort on grip strength tests.

US DEPARTMENT OF LABOR – DESCRIPTIONS OF PHYSICAL DEMAND LEVELS OF WORK

(From the "Components of Definition Trailer" Dictionary of Occupational Titles, Appendix C., 4th Edition. Revised 1991. Pages 1012-1013).

Sedentary Work - Exerting up to 10 pounds of force occasionally (Occasionally: activity or condition exists up to 1/3 of the time) and/or a negligible amount of force frequently (Frequently: activity or condition exists from 1/3 to 2/3 of the time) to lift, carry, push, pull, or otherwise move objects, including the human body. Sedentary work involves sitting most of the time, but may involve walking or standing for brief periods of time. Jobs are sedentary if walking and standing are required only occasionally and all other sedentary criteria are met. *See note below

Light Work - Exerting up to 20 pounds of force occasionally, and/or up to 10 pounds of force frequently, and/or a negligible amount of force constantly (Constantly: activity or condition exists 2/3 or more of the time) to move objects. Physical demand requirements are in excess of those for Sedentary Work. Even though the weight lifted may be only a negligible amount, a job should be rated Light Work: (1) when it requires walking or standing to a significant degree; or (2) when it requires sitting most of the time but entails pushing and/or pulling of arm or leg controls; and/or (3) when the job requires working at a production rate pace entailing the constant pushing and/or pulling of materials even though the weight of those materials is negligible. NOTE: The constant stress and strain of maintaining a production rate pace, especially in an industrial setting, can be and is physically demanding of a worker even though the amount of force exerted is negligible. *See note below

Medium Work - Exerting 20 to 50 pounds of force occasionally, and/or 10 to 25 pounds of force frequently, and/or greater than negligible up to 10 pounds of force constantly to move objects. Physical Demand requirements are in excess of those for Light Work.

Heavy Work - Exerting 50 to 100 pounds of force occasionally, and/or 25 to 50 pounds of force frequently, and/or 10 to 20 pounds of force constantly to move objects. Physical Demand requirements are in excess of those for Medium Work.

Very Heavy Work - Exerting in excess of 100 pounds of force occasionally, and/or in excess of 50 pounds of force frequently, and/or in excess of 20 pounds of force constantly to move objects. Physical Demand requirements are in excess of those for Heavy Work.

*Note - The Department of Labor in SSR 83-10 further defined the Sedentary level of work to require sitting frequently with occasional walking or standing and the Light level work to require frequent standing OR frequent walking plus occasional sitting.

US DICTIONARY OF OCCUPATIONAL TITLES – TASK FREQUENCY/DURATION TERMS

Constantly: activity or condition exists 2/3 or more of the time

Frequently: activity or condition exists from 1/3 to 2/3 of the time

Occasionally: activity or condition exists up to 1/3 of the time

US DICTIONARY OF OCCUPATIONAL TITLES – DEXTERITY APTITUDE LEVELS

The Dictionary of Occupational Titles (DOT), as published by the US Department of Labor, expresses finger and manual dexterity requirements of occupations as an Aptitude Score using a 1-5 scale:

1 = top 10% of population (90-100 percentile score)

2 = highest 1/3, exclusive of top 10% (67-89 percentile score)

3 = middle 1/3 (34-66 percentile score)

4 = lowest 1/3, exclusive of bottom 10% (11-33 percentile score)

5 = lowest 10% of population (0-10 percentile score)

CLIENT PARTICIPATION DEFINITIONS

Appropriate: Client and therapist agree on stopping task. Full physical effort given.

Overextending: Therapist stops task. Client willing to continue despite maximum effort being reached. Full, physical effort given.

Self-limiting: Client stops task before objective signs indicate that maximum physical effort has been reached. Performance reflects what client is willing to do.

SUMMARY OF ERGOSCIENCE PWPE RESEARCH & CITATIONS

This report summarizes the results of the ErgoScience FCE Physical Work Performance Evaluation TM. The Physical Work Performance Evaluation by ErgoScience is a standardized and objective method for Functional Capacity Evaluation which is supported by scientifically recognized, peer reviewed research that documents its reliability and validity. No other FCE protocol has this level of research supporting the consistency and accuracy of the test method.

1. Lechner et al "Reliability and Validity of a Newly Developed Test of Physical Work Performance – JOM – Vol 36, Number 9, Sept 1994

2. Lechner, D.E., Page, J. J., Sheffield, G. – "Predictive Validity of a Functional Capacity Evaluation: The Physical Work Performance Evaluation" - Work 30 (2008)

3. Tuckwell, N., Starker, L., & Barrett, T – "Test-retest reliability on nine tasks of the Physical Work Performance Evaluation" - Work 19, 243-253, 2002.

4. Durand, Loisel et al – "The inter-rater reliability of a Functional Capacity Evaluation" – The Physical Work Performance Evaluation" – Journal of Occupational Rehabilitation – Vol 14, Number 2, June 2004.

5. Stokes et al – "The Identification of Low-effort Patients through Dynamometry" – Journal of Hand Surgery – Vol 20A, Number 6, Nov 1995

6. Lechner et al "Detecting Sincerity of Effort" – Physical Therapy – Vol 78, Number 8, Aug 1998

Musculoskeletal assessment

Range of Motion		
Knee	Right	Left
Flexion	135 degrees	135 degrees
Extension	0 degrees	0 degrees
Hip	Right	Left
Flexion	120 degrees	120degrees
Extension	30 degrees	30 degrees
Internal Rotation	25 degrees	25 degrees
External Rotation	45 degrees	60 degrees
Shoulder		
Flexion	180 degrees	180 degrees
Extension	60 degrees	55degrees
Abduction	180 degrees	180 degrees
Internal Rotation	80 degrees	75d degrees
External Rotation	80 degrees	80 degrees
Elbow		
Flexion	150 degrees	145 degrees
Extension	0 degrees	0 degrees
Forearm Pronation	80 degrees	80 degrees
Forearm Supination	90 degrees	90 degrees
Wrist		
Flexion	80 degrees	75 degrees
Extension	80 degrees	75degrees
Cervical Spine – measured with inclinometer at crown of head and C7 spinous process		
Flexion	40 degrees	
Extension	35 degrees	
Right Sidebend	20 degrees	
Left Sidebend	20 degrees	
Lumbar Spine – measured with inclinometer at T12 and sacrum		
Flexion	50 degrees	
Extension	20 degrees	
Right Sidebend	15 degrees	
Left Sidebend	15 degrees	
Strength: MMT		
Knee	Right	Left
Flexion	5/5	5/5
Extension	4/5	4/5
Hip	Right	Left
Flexion	4+/5	4+/5
Extension	4/5	4/5
Abduction	4/5	4/5
Adduction	5/5	5/5
Internal Rotation	5/5	5/5
External Rotation	5/5	5/5
Shoulder	Right	Left
Flexion	5/5	4/5
Abduction	5/5	4+/5
Internal Rotation	5/5	4/5
External Rotation	5/5	4/5
Elbow	Right	Left
Flexion	5/5	4+/5

Extension	5/5	4/5
Wrist	Right	Left
Flexion	5/5	4/5
Extension	5/5	4+/5
Grip Strength	Right – measured in pounds	Left – measured in pounds
Standard grip	28	24
Sensation	Right	Left
Light Touch		
Upper Extremity Dermatomes	Intact	Intact
Lower Extremity Dermatomes	Intact	Intact
Sharp Touch		
Upper Extremity Dermatomes	Intact	Intact
Lower Extremity Dermatomes	Intact	Intact
Deep Tendon Reflexes	Right	Left
Biceps	2+	2+
Triceps	2+	2+
Patella	2+	2+
Achilles	2+	2+
Neural Dynamics	Right	Left
Sciatic	Positive	Positive
Median	Positive	Positive
Ulnar	negative	negative
Radial	negative	Negative

BenchMark REHAB PARTNERS

Date: 09/19/2022

Patient Name: Viktoriya Pulliam

Patient Account #: 2301896

Date of Birth: 12/06/1989

Date of Service: 09/22/2022

Attorney: Brandon Squires

A Physical Capacity Evaluation (PCE)/Functional Capacity Evaluation (FCE) is scheduled to be performed for the client listed above. Reporting will be sent directly to you at the conclusion of each assessment. The billing form will be sent following the completion of the FCE. Services will be provided at a BenchMark Physical Therapy Facility in one of the following locations:

BenchMark Physical Therapy – Lake Oswego
440 1st Street
Lake Oswego OR 97034

BenchMark Physical Therapy – Sherwood
16310 SW Langer Drive
Sherwood OR 97140

BenchMark Physical Therapy – Woodburn
1537 Mt Hood Ave, Suite 103
Woodburn OR 97071

BenchMark Physical Therapy – Lancaster
4132 Devonshire Ct NE
Salem OR 97305

BenchMark Physical Therapy – Lincoln City
3007 NE West Devils Lake Rd
Lincoln City OR 97367

BenchMark Physical Therapy – Eugene
2490 Willamette Street Suite 5
Eugene OR 97405

The price of the PCE/FCE is $800 and due on or before the time of service. A scheduled PCE/FCE must be cancelled 72 hours prior to the scheduled appointment to avoid being charged. A fee of $400 will be charged for an incomplete PCE/FCE not related to a medical reason. A fee of $200 will be charged for any No Show appointments. All fees incurred are

expected to be paid within 15 business days of the original scheduled appointment.

I have verified that the above information is accurate. If there are any modifications or additional services to be provided by BenchMark Rehab Partners, then primary contact listed above will notify BMRP and additional services will be added to this statement of work at their request. **Payment is due on or before the time of service. Failure to comply/pay the outstanding balance will result in suspension of future services.** Bills in the form of a HCFA 1500 are sent to _____ within 30 days of date of service.

DocuSigned by:
Brandon J. Squires 5/9/2023
F131514638E14E0...

Attorney Signature Date

BenchMark REHAB PARTNERS

EXECUTIVE SUMMARY

Name: Victoriya Pulliam	**D.O.B.: 12/06/1989**	**Evaluation Date:** 06/05/2023

PURPOSE OF FCE: Determine current abilities, compare abilities to own job /the position of Regional Administrator, and suggest restrictions.

EFFORT LEVEL:

Self-limiting participation (sub-maximal effort): 5% of tasks.

Full participation (exerted maximal effort) on: 95% of tasks.

STOKES CONSISTENCY OF EFFORT TESTING: Very weak evidence of low effort and inconsistent behavior.

Tasks	Client Performance [1]	Self-Limiting Reasons	Maximum Abilities [1]	Job Demand D.O.T.	Match D.O.T.
Floor to waist lift	38 lb Occas.		38 lb Occas.	0	NA
Waist to eye level lift	38 lb Occas.		38 lb Occas.	0	NA
Two handed carrying	26 lb Occas.		26 lb Occas.	0	NA
One handed carrying	*L20 R20 lb-SL*	Pain	*Unable to Determine*	0	NA
Pushing	45 lb Occas. [3]		45 lb Occas. [3]	0 [3]	NA
Pulling	40 lb Occas. [3]		40 lb Occas. [3]	0 [3]	NA
Sitting	Constantly		Constantly	Constantly	Yes
Standing	Never [4]		Never	Occasionally	No
Work arms over head-standing	Never		Never	Occasionally	No
Work bent over-standing/stooping	Never		Never	Occasionally	No
Work kneeling	Frequently		Frequently	Never	NA
Work bent over-sitting	Occasionally		Occasionally	Occasionally	Yes
Work squatting/crouching	Occasionally		Occasionally	Occasionally	Yes
Work arms over head-supine	Frequently		Frequently	Never	NA
Climbing stairs	Frequently		Frequently	Occasionally	Yes
Repetitive squatting	Frequently		Frequently	Never	NA
Walking	Frequently		Frequently	Occasionally	Yes
Crawling	Frequently		Frequently	Never	NA
Climbing a ladder	Frequently		Frequently	Never	NA
Repetitive trunk rotation-sitting	Frequently		Frequently	Occasionally	Yes
Repetitive trunk rotation-standing	Frequently		Frequently	Occasionally	Yes
Balance on level surfaces	Adequate		Adequate	Not Required	NA
Balance on uneven surfaces	Adequate		Adequate	Not Required	NA
Balance on ladder	Adequate		Adequate	Not Required	NA
Grip Strength	L40 R42 lb		L40 R42 lb		

CONCLUSIONS/SUMMARY

Capacity/Endurance: Overall Level of work: Demonstrated ability to work **within the Medium** Overall Level.

Material Handling Strength Classification: Medium

Endurance: Demonstrated **adequate** endurance to perform work on a full time basis.

Per the **employer's description/self-report** of the job demands, the following tasks were not met: standing, working with arms overhead standing, and working bent over/stooping.

Suggested Restrictions:

- It may not be appropriate to provide restrictions for tasks with self-limiting performance including one handed carry,therefore, only the tasks in which a maximum ability was demonstrated may be considered for assigning restrictions.
- Material handling: 12-24" lifting surface for loads up to 25# on the occasional basis, two hand carry up to 20# on the occasional basis; standing limited to short bouts throughout the day.

For additional details, please refer to the full report attached. If there are any questions about this test, please do not hesitate to call.

Sincerely,

Melissa A. Clay, PT, DPT, COMT, ATC

503.744.4826

Physical Work Performance Evaluation™

BenchMark Physical Therapy
440 1st Street, Lake Oswego OR 97034
Phone 503-744-4826 Fax 503-744-4828

Name: Pulliam, Vicktoriya
Injury/Onset Date: 11/21/2019
Evaluation Date: 06/05/2023
Test Start, End, Duration: 7:46 AM, 11:52 AM, 4:06 hours
Diagnosis:
Height, Weight: 5'5", 166lb
Starting BP, HR, Pain: 126/92, 71 bpm, Pain 0 out of 10

This report summarizes the results of the ErgoScience FCE **Physical Work Performance Evaluation™**. This evaluation is substantiated by reliability and validity research conducted at the University of Alabama at Birmingham and reported in the *Journal of Occupational Medicine*, September 1994[1]

Overall Level of Work: Falls within the Medium range. Exerting 20 to 50 pounds of force occasionally, and/or 10 to 25 pounds of force frequently, and/or greater than negligible up to 10 pounds of force constantly to move objects. Physical Demand requirements are in excess of those for Light Work. **Please see the Task Performance Table for specific abilities.**

Tolerance for the 8-Hour Day: Based on this evaluation, the client is capable of sustaining the Medium level of work for an 8-hour day/40-hour week.

Self Limiting Behavior: The client self-limited on 5% of the 21 tasks. Self-limiting behavior means that the client stopped the task before a maximum effort was reached. Possible causes of self-limiting behavior include: (1) pain; (2) psychosocial issues such as fear of reinjury, anxiety, or depression; and/or (3) attempts to manipulate test results. Although it is difficult to determine the causes of self-limiting behavior, our research indicates that motivated clients self-limit on no more than 20% of test items. If the self-limiting exceeds 20%, then psychosocial and/or motivational factors are affecting test results.

- **Self Limiting < 20% of tasks = Within normal limits[1]**
- Self-Limiting 21% to 33% of tasks = Exceeds normal limits[1]
- Self-Limiting > 33% of tasks = Significantly exceeds normal limits[1]

[1]When compared to a motivated group of patients who participated in research.

[1] Lechner DE, et al. *Journal of Occupational Medicine.* September 1994 Volume 36, No. 9: pages 997-1004.

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ADDITIONAL CLIENT STATEMENTS ABOUT SELF-LIMITING BEHAVIOR

The client's stated reasons for self-limiting behavior are listed beside each self-limited task in the Task Performance Summary section of this report. In addition, the client made the following statements about self-limiting behavior during the evaluation:

- during squat portion

RESULTS OF FORMAL CONSISTENCY OF EFFORT TESTING

- The ErgoScience FCE utilizes a formal consistency of effort protocol established and validated by Stokes et al.[2] In this protocol, three (3) different statistical calculations on grip strength testing data are performed. These results are then combined with any evidence of clinical inconsistencies or self-limiting behavior observed during the ErgoScience FCE. The final consistency of effort conclusion indicates the strength of all of this evidence combined.
- Combining the results of the clinical consistency comparisons, the presence of self-limiting behavior and the three formal consistency cross comparisons of the grip strength data, indicates that there is very weak evidence of low effort and inconsistent behavior. See addendum for details of the formal consistency of effort scoring criteria.

SUBJECTIVE PAIN STATEMENTS

The client made the following subjective pain statements during the test:

- -This test is hard because I don't want to be crippled later caring for my daughter
- -It (back) is stiff
- -It is starting to shoot into my hip more
- -It hurts more when I am lifting
- -The squatting is painful
- These pain statements were consistent with the observed movement patterns.

PAIN BEHAVIORS AND THEIR IMPACT ON TEST RESULTS

The client demonstrated the following pain behaviors during the test:

- -Bending forward, unweighting lumbar spine
- -Holding back
- -Tilting head/rotating head
- -Shrugging shoulders
- These pain behaviors were consistent with the observed movement patterns.
- These pain behaviors correlated with the client's self-reported pain.
- These pain behaviors did not affect test performance

OTHER EXTERNAL FACTORS THAT MIGHT IMPACT TEST RESULTS

- Gave birth to daughter on 03/24/2023; Job description was cross referenced with similar DOT job descriptions due to lack of material handling requirements - unable to locate specific job description - both jobs cross referenced denoted Sedentary PDL levels (184.161.014, 184.117.054)

BODY MECHANICS AND MOVEMENT PATTERNS

The client did not demonstrate safe body mechanics and movement patterns during the test. Examples of the unsafe body mechanics and movement patterns are:

- Bending forward at hips while lifting/decreased squat position
- The client could benefit from further instructions in safe body mechanics.

BRIEF SUMMARY OF MEDICAL HISTORY

MOI: First MVA 11/21/2019 - pt. driving and was hit on the driver's side tire from someone that ran a stop sign; Second MVA 02/11/20 - pt. driving and was hit on the passenger's side tire area from someone that ran a red light
Hx of present injury: urgent care, chiropractic, massage therapy, PT, dry needling, pain management, Lumbar spine ablations, epidural injections, C5-7 fusion
PMHX: post partum 03/24/2023
Current symptoms: tightness, soreness, pain, occasional sciatica

Work status: full time/full duty - desk/computer work

BRIEF MUSCULOSKELETAL SCREEN		
Range of Motion		
Shoulder		
Flexion	152 degrees	147 degrees
Extension	48 degrees	46 degrees
Abduction	165 degrees	162 degrees
Internal Rotation	70 degrees	70 degrees
External Rotation	86 degrees	86 degrees
Cervical Spine – measured with inclinometer at crown of head and C7 spinous process		
Flexion	29 degrees	
Extension	58 degrees	
Right Sidebend	27 degrees	
Left Sidebend	24 degrees	
Lumbar Spine – measured with inclinometer at T12 and sacrum		
Flexion	54 degrees – Gower's sign, pain/stiffness	
Extension	10 degrees – pain/stiffness	
Right Sidebend	12 degrees	
Left Sidebend	16 degrees	
Strength: Myotomes		
Ankle	Right	Left
Great toe ext	5/5	5/5
Dorsiflexion	5/5	5/5
Plantarflexion	3+/5	4-/5
Knee	Right	Left
Flexion	5/5	5/5
Extension	5/5	5/5
Hip	Right	Left
Flexion	5/5	5/5
Shoulder	Right	Left
Flexion	5/5	5/5
Abduction	5/5	5/5
Internal Rotation	5/5	5/5
External Rotation	5/5	5/5
Elbow	Right	Left
Flexion	5/5	5/5
Extension	5/5	5/5
Wrist/Hand	Right	Left
Flexion	5/5	5/5
Extension	5/5	5/5
Thumb extension	5/5	5/5
Lumbricales	5/5	5/5
Sensation	Right	Left
Light Touch		
Upper Extremity	Intact	Intact

Dermatomes		
Lower Extremity Dermatomes	Intact	Intact
Sharp Touch		
Upper Extremity Dermatomes	Intact	Intact
Lower Extremity Dermatomes	Intact	Intact
Deep Tendon Reflexes	Right	Left
Biceps	WNL	WNL
Triceps	WNL	WNL
Patella	WNL	WNL
Achilles	WNL	WNL
Neural Dynamics	Right	Left
Sciatic	Negative	Negative
Femoral	Negative	Positive
Median	Negative	Adverse neural dynamics
Ulnar	Negative	Negative
Radial	Negative	Negative
Slump	Negative	Negative

TEST LENGTH AND REST BREAKS

The test lasted 4:06 hours. Short pauses of 2-3 minutes between tasks occur while the evaluator is setting up equipment and documenting scores. No additional rest breaks were taken.

TASK PERFORMANCE

Please note that the results noted below in red italics and marked SL are tasks on which the client self-limited. On these self-limiting tasks, the performance represents a minimal rather than a maximal performance. On these tasks we cannot determine a maximum performance.

Tasks	Client Performance [1]	Self-Limiting Reasons	Maximum Abilities [1]	Job Demand D.O.T.	Match D.O.T.
Floor to waist lift	**38 lb Occas.**		**38 lb Occas.**	**0**	**NA**
Waist to eye level lift	**38 lb Occas.**		**38 lb Occas.**	**0**	**NA**
Two handed carrying	**26 lb Occas.**		**26 lb Occas.**	**0**	**NA**
One handed carrying	***L20 R20 lb-SL***	**Pain**	***Unable to Determine***	**0**	**NA**
Pushing	**45 lb Occas.** [3]		**45 lb Occas.** [3]	**0** [3]	**NA**
Pulling	**40 lb Occas.** [3]		**40 lb Occas.** [3]	**0** [3]	**NA**
Sitting	**Constantly**		**Constantly**	**Constantly**	**Yes**
Standing	**Never** [4]		**Never**	**Occasionally**	**No**
Work arms over head-standing	**Never**		**Never**	**Occasionally**	**No**
Work bent over-standing/stooping	**Never**		**Never**	**Occasionally**	**No**
Work kneeling	**Frequently**		**Frequently**	**Never**	**NA**
Work bent over-sitting	**Occasionally**		**Occasionally**	**Occasionally**	**Yes**
Work squatting/crouching	**Occasionally**		**Occasionally**	**Occasionally**	**Yes**
Work arms over head-supine	**Frequently**		**Frequently**	**Never**	**NA**
Climbing stairs	**Frequently**		**Frequently**	**Occasionally**	**Yes**
Repetitive squatting	**Frequently**		**Frequently**	**Never**	**NA**
Walking	**Frequently**		**Frequently**	**Occasionally**	**Yes**
Crawling	**Frequently**		**Frequently**	**Never**	**NA**
Climbing a ladder	**Frequently**		**Frequently**	**Never**	**NA**
Repetitive trunk rotation-sitting	**Frequently**		**Frequently**	**Occasionally**	**Yes**
Repetitive trunk rotation-standing	**Frequently**		**Frequently**	**Occasionally**	**Yes**
Balance on level surfaces	**Adequate**		**Adequate**	**Not Required**	**NA**
Balance on uneven surfaces	**Adequate**		**Adequate**	**Not Required**	**NA**
Balance on ladder	**Adequate**		**Adequate**	**Not Required**	**NA**
Grip Strength	**L40 R42 lb**		**L40 R42 lb**		

1 Occasionally = up to 1/3 of the day, Frequently = 1/3 to 2/3 of the day, Constantly = 2/3 to the full day. Frequent lifting = 50% of Occasional; Constant lifting = 20% of Occasional.

2 D.O.T. The aptitudes: **1** (90-100 percentile), **2** (67-89 percentile), **3** (34-66 percentile), **4** (11-33 percentile), **5** (0-10 percentile).

3 Pounds of force is the force used with a force gauge.

4 A never score for standing does not mean that the client literally can never stand. The client may be able to stand for brief periods but is not able to sustain this for up to 1/3 of the day as required for the "Occasional" category.

MAJOR AREAS OF DYSFUNCTION

- Dynamic Strength

FACTORS UNDERLYING PERFORMANCE

- Decreased muscle strength in Lower quarter with squatting
- Decreased range of motion in lumbar spine, cervical spine
- Generalized de-conditioning
- Pain in lumbar spine and gluteal regions
- Neural dynamics

POSSIBLE INTERVENTIONS

- The following approaches may be helpful for increasing the client's overall work level:
    - Improve client's overall physical condition by 50% through physical therapy/exercise program
    - Improve client's strength in lower quarter muscle groups by 50% through physical therapy/exercise program
    - Improve client's range of motion in lumbar spine joints by 50% through physical therapy/exercise program
    - Improve pain control techniques through pain management and/or physical therapy
    - Client may be able to return to a former job/occupation if the following modifications are made to work tasks, station, schedule, practices, or environment if feasible: Currently able to perform job duties. If standing is required, modifications to short bouts throughout the day. If, in the future the patient returns to work in the office setting and material handling is required to carry out job duties, modifications required: 12-24" lifting surface for loads up to 25# on the occasional basis, two hand carry up to 20# on the occasional basis

EXIT INTERVIEW

- Changes in Musculoskeletal Status from beginning to end: Feels stiff and more painful; a little faint
- Pain Score: 3
- Gait pattern leaving the evaluation is a little worse compared to gait pattern used upon arriving for test.
- The client drove herself to the evaluation.
- 126/96 BP
- 74 HR

Evaluator: Melissa A Clay PT, DPT, COMT, ATC
Phone: 503-744-4826

Consistency of Effort Testing and Conclusion

The ErgoScience FCE utilizes a formal consistency of effort protocol established and validated by Stokes et al.[2] In this protocol, three (3) different statistical calculations on grip strength testing data are performed. These results are then combined with any evidence of clinical inconsistencies or self-limiting behavior observed during the ErgoScience FCE. The final consistency of effort conclusion indicates the strength of all of this evidence combined.

Statistical Test 1.
Right Hand
The standard deviation of sustained maximum right grip strength across 5 handle positions was 3.16, indicating a relatively flat bell shaped curve and low effort on the bell shaped curve test for the right hand.[2]

Left Hand
The standard deviation of sustained maximum left grip strength across 5 handle positions was 4.90, indicating a relatively flat bell shaped curve and low effort on the bell shaped curve test for the left hand.[2]

Statistical Test 2.
The Rapid Exchange Grip (REG) is 10 pounds different from the peak slow sustained grip. Clinical studies demonstrate[2] that this difference is not significant and indicates that the client exerted maximum effort.

Statistical Test 3.
A regression analysis was calculated based on the peak effort of sustained grip and the maximum REG. This calculation indicates that the patient gave a maximum effort on grip strength tests.[2]

Self-Limiting Behavior
Self-Limiting behavior was ≤ 20%.

Clinical Inconsistencies
No additional significant clinical inconsistencies were noted during the FCE.

Conclusion Regarding Consistency of Effort. Combining the results of the clinical consistency comparisons, the presence of self-limiting behavior and the three formal consistency cross comparisons of the grip strength data, indicates that there is **very weak** evidence of low effort and inconsistent behavior.[2]

[2]Stokes, HM et al. Identification of Low-effort Patients Through Dynamometry. Journal of Hand Surgery. Vol 20A, No 6, November, 1995, pp. 1047 – 1055.

Conditions for Positive Results (indicating low effort) on Statistical Calculations:

1) SD of 5-position grip testing on right or left grip of ≤ 7.5
2) Difference between REG and peak grip of 5-position grip testing of ≥ 12 lb.
3) Regression equation results of ≥ 3.5

Criteria for Consistency of Effort Conclusion:

1) Significant clinical inconsistencies ***or*** SL behavior [2] present + all 3 statistical calculations are positive = **Very Strong evidence** of low effort and inconsistent behavior.
2) Significant clinical inconsistencies ***or*** SL behavior [2] present + 2 of the 3 statistical calculations are positive = **Strong evidence** of low effort and inconsistent behavior.
3) Significant clinical inconsistencies ***or*** SL behavior [2] present + 1 of the 3 statistical calculations are positive = **Significant evidence** of low effort and inconsistent behavior.

3a) Significant clinical inconsistencies present + 0 (none) of the 3 statistical calculations are positive = **Significant evidence** of low effort and inconsistent behavior.

4) No significant clinical inconsistencies ***and*** no SL behavior [2] present + all 3 statistical calculations are positive = **Moderate evidence** of low effort and inconsistent behavior.
5) No significant clinical inconsistencies ***and*** no SL behavior [2] present + 2 of the 3 statistical calculations are positive = **Weak evidence** of low effort and inconsistent behavior.
6) No significant clinical inconsistencies ***and*** no SL behavior [2] present + 1 of the 3 statistical calculations are positive = **Very Weak evidence** of low effort and inconsistent behavior.

6a) No significant clinical inconsistencies ***but*** SL behavior [2] present + 0 (none) of the 3 statistical calculations are positive = **Very Weak evidence** of low effort and inconsistent behavior.

7) No significant clinical inconsistencies ***and*** no SL behavior [2] present + 0 (none) of the 3 statistical calculations are positive = **No evidence** of low effort and inconsistent behavior. Patient gave full/maximum effort on all aspects of the test.

DATA DETAILS

Note: For each task, client participation is rated as:

Appropriate – Client and therapist agree on stopping task. Full, physical effort given.

Overextending – Therapist stops task. Client willing to continue despite maximum being reached. Full, physical effort given.

Self-limiting – Client stops task before objective signs indicate that a maximum physical effort has been reached.

Lift - Floor to Waist - Over-extending

- Completed 38 lb safely
- Pain Score = 2.00
- Pain Location = Lower back R>L
- Ending HR = 90

Signs of Effort

- Accessory Muscles
- Post Trunk Lean
- Decreased Box Control
- Increased Time to Complete Repetitions
- Vertical Trunk Alignment Decreases
- Props Box on Thigh
- Irregular Steps
- Increased Thoracic Kyphosis with Protraction of the Shoulder Girdle

Climbing Stairs - Appropriate

- Completed 100 of 100 - 100% of task
- Pain Score = 2.00
- Pain Location = Lower back
- Ending HR = 100
- Minimal Deviations
  - o Decreased Hip Extension

o Decreased Knee Extension

Repetitive Squatting - Appropriate

- Completed 25 of 25 - 100% of task
- Pain Score = 3.00
- Pain Location = Lower back
- Ending HR = 112
- Moderate Deviations
  o Loss of Vertical Trunk Alignment

Lift - Waist Height to Eye Level - Appropriate

- Completed 38 lb safely
- Pain Score = 1.00
- Pain Location = Lower back
- Ending HR = 95

Signs of Effort

- Accessory Muscles
- Post Trunk Lean
- Hands Slip/Difficulty Holding Box
- Decreased Box Control
- Upper Trap Elevation
- Box Resting Against Chest
- Increased Knee Flexion in Preparation for Lift

Bilateral Carry - Appropriate

- Completed 26 lb safely
- Pain Score = 3.50
- Pain Location = Lower back/hips
- Ending HR = 100

Signs of Effort

- Face Red/Perspiration
- Accessory Muscles
- Post Trunk Lean
- Irregular Gait

Unilateral Carry - Self-Limiting

- Left completed 20 lb safely Right completed 20 lb safely
- Pain Score = 4.00
- Pain Location = Lower back/hips
- Ending HR = 97

Signs of Effort

- Accessory Muscles
- Lateral Trunk Lean
- Thigh Supporting Toolbox
- Irregular Gait
- Rapid Alternating Trunk Shift

Maximum Dynamic Pushing - Appropriate

- Completed 45 lb safely
- Pain Score = 3.00
- Pain Location = Lower back/hips
- Ending HR = 89

Signs of Effort

- Max effort w/ three trials w/ force gauge

Maximum Dynamic Pulling - Appropriate

- Completed 40 lb safely
- Pain Score = 3.00
- Pain Location = Lower back/hips
- Ending HR = 85

Signs of Effort

- Max effort of three trials with force gauge

Sitting Tolerance - Appropriate

- Completed 5:00 of 5:00 minutes - 100% of task
- Pain Score 1 Min = 2
- Pain Location 1 Min = Lower back/hips
- Within Normal Limits

Standing Tolerance - Appropriate

- Completed 5:00 of 5:00 minutes - 100% of task
- Position Adjustments = 9
- Pain Score 1 Min = 3
- Pain Location 1 Min = Lower back/hips
- Severe Deviations
  - o Decreased Weight Bearing on One Leg
  - o Lateral trunk Shift (Right or Left)
  - o Increased Hip and Knee Flexion (Bilateral or Unilateral)

Work Arms Overhead & Standing - Appropriate

- Completed 5:00 of 5:00 minutes - 100% of task
- Position Adjustments = 15
- Pain Score 1 Min = 4 3 Min = 5 5 Min = 6
- Pain Location 1 Min = Neck/back/hips 3 Min = Neck/back/hips 5 Min = Neck/back/hips
- Severe Deviations
  - o Decreased Weight Bearing on One Leg
  - o Lateral Trunk Shift (Right or Left)
  - o flexing head/back; tilting head side to side; dropping arms

Work Bent Over / Stooping - Appropriate

- Completed 5:00 of 5:00 minutes - 100% of task
- Position Adjustments = 10
- Pain Score 1 Min = 3 3 Min = 5 5 Min = 6
- Pain Location 1 Min = Lower back/hips 3 Min = Lower back/hips 5 Min = Lower back/hips
- Severe Deviations
  - o Decreased Weight Bearing on One Leg
  - o Lateral Trunk Shift (Right or Left)
  - o Increased Hip and Knee Flexion (Bilateral or Unilateral)
  - o straightening back out of crouched position

Kneeling - Appropriate

- Completed 5:00 of 5:00 minutes - 100% of task
- Position Adjustments = 2
- Pain Score 1 Min = 5 3 Min = 5 5 Min = 5
- Pain Location 1 Min = Lower hips 3 Min = Lower hips 5 Min = Lower hips/neck/upper back
- Minimal Deviations
  - o Increased Hip Flexion with Anterior Pelvic Tilt
  - o Weight Bearing with Hand
  - o tilting/turning head

Work Bent Over Sitting - Appropriate

- Completed 5:00 of 5:00 minutes - 100% of task

- Position Adjustments = 5
- Pain Score 1 Min = 2 3 Min = 2 5 Min = 3
- Pain Location 1 Min = Lower back/neck/shoulders 3 Min = Lower back/neck/shoulders 5 Min = Lower back/neck/shoulders
- Moderate Deviations
  - o Supporting Body Weight through Legs
  - o Scapular Protraction
  - o tilting head back and forth/shrugging shoulders

Squatting - Appropriate

- Completed 5:00 of 5:00 minutes - 100% of task
- Position Adjustments = 6
- Pain Score 1 Min = 3 3 Min = 4 5 Min = 3
- Pain Location 1 Min = Neck 3 Min = Neck/lower hips 5 Min = Neck/lower hips
- Moderate Deviations
  - o Excessive Hip Abduction and External Rotation
  - o Difficulty Balancing
  - o tilting head back and forth; bearing weight through UEs on shelf

Work Arms Overhead Supine - Appropriate

- Completed 5:00 of 5:00 minutes - 100% of task
- Position Adjustments = 4
- Pain Score 1 Min = 2 3 Min = 1 5 Min = 1
- Pain Location 1 Min = Hips/neck 3 Min = Hips/neck 5 Min = Hips/neck
- Minimal Deviations
  - o Rolling Trunk from Side to Side to Substitute for Shoulder ROM
  - o nodding head

Walking - Appropriate

- Completed 500 of 500 - 100% of task
- Pain Score = 2.00
- Pain Location = R hip
- Ending HR = 132
- Minimal Deviations
  - o Increased pelvis rotation B; increased lumbar extension; decreased R arm swing

Crawling - Appropriate

- Completed 50 of 50 - 100% of task
- Pain Score = 2.00
- Pain Location = Hips/neck
- Ending HR = 123
- Within Normal Limits

Climbing a Ladder - Appropriate

- Completed 40 of 40 - 100% of task
- Pain Score = 3.00
- Pain Location = R hip
- Ending HR = 109
- Minimal Deviations
  - o Takes one Step at a Time Rather than One Foot over the Other
  - o See-Saw Motion of Trunk

Repetitive Trunk Rotation - Sitting - Appropriate

- Completed 25 of 25 - 100% of task
- Pain Score = 2.00
- Pain Location = Neck/shoulders

- Ending HR = 93
- Minimal Deviations
  o Decreased Elbow Extension

Repetitive Trunk Rotation - Standing - Appropriate

- Completed 25 of 25 - 100% of task
- Pain Score = 2.00
- Pain Location = Neck
- Ending HR = 105
- Minimal Deviations
  o Decreased Elbow Extension

Isometric Grip Strength - Appropriate

- Pain Location = NA
- Percentile Score Left = 10% Right = 0%
- Grip Avg. Left = 40 lb Right = 42 lb
- Within Normal Limits

BenchMark REHAB PARTNERS

PCE Level 1 Evaluation

BenchMark Physical Therapy	Lake Oswego	
Phone 503.744.4826 Fax 503.744.4828		Service Date: 09 /22/2022
PATIENT'S NAME Viktoriya Pulliam	**DOB** 12/06/1989	**DOI** 11/21/2019 AND 02/11/2020
PROVIDER NAME *Melissa A. Clay, PT, DPT, OCS, COMT, ATC*		
DIAGNOSIS *Cervical Spine/Lumbar Spine pain*	**PHYSICIAN'S NAME** N/A	

SUBJECTIVE HISTORY

History of Present Injury: MVA November 2019 and February 2020, numbness tingling B UEs L>R, cervical pain, lumbar pain
Current Symptoms: stiffness, tightness
Past Medical / Surgical History: chiropractic care cervical/lumbar, massage therapy cervical/lumbar, lumbar ablation, physical therapy cervical, ant cervical fusion – 2 level, physical therapy following surgery
Work Status: full time, full duty
Prior injury to involved and uninvolved side: N/A

PHYSICAL EXAMINATION

Observations: anterior right cervical scar 6 cm; able to perform ½ squat – limited B hip ROM, ankle ROM
Palpation: Mod TTP B UT, B cervical/thoracic/lumbar paraspinals; scar mildly adherent at distal aspect

Range of Motion		
Cervical Spine – measured with inclinometer at crown of head and C7 spinous process		
Flexion	40 degrees	
Extension	48 degrees	
Right Sidebend	30 degrees	
Left Sidebend	35 degrees	
Right rotation	62 degrees (measured with goniometer)	
Left rotation	50 degrees (measured with goniometer	
Lumbar Spine – measured with inclinometer at T12 and sacrum		
Flexion	50 degrees	
Extension	10 degrees	
Right Sidebend	15 degrees	
Left Sidebend	17 degrees	
Posture		
Mildly rounded shoulders, mildly increased Lumbar lordosis		
Gait		
WFL		
Strength: Myotomes		
Ankle	Right	Left
Dorsiflexion	5/5	5/5
Plantarflexion	5/5	5/5
Knee	Right	Left
Flexion	5/5	5/5
Extension	5/5	5/5
Hip	Right	Left

Flexion	5/5				5/5			
Shoulder	Right				Left			
Flexion	4/5				4/5			
Abduction	5/5				5/5			
Internal Rotation	5/5				5/5			
External Rotation	5/5				5/5			
Elbow	Right				Left			
Flexion	4+/5				5/5			
Extension	5/5				5/5			
Wrist	Right				Left			
Flexion	4/5				5/5			
Extension	5/5				5/5			
Thumb extension	5/5				5/5			
Lumbrical interossei	5/5				5/5			
Grip Strength	Right – measured in pounds				Left – measured in pounds			
	Tr 1	Tr 2	Tr 3	**Avg**	Tr 1	Tr 2	Tr 3	**Avg**
Hand	45	45	50	**46.7**	45	45	45	**45**
Tip	5	5	4	**4.7**	5	5	4	**4.7**
Lateral	13	11	11	**11.7**	11	11	11	**11**
Three Jaw Chuck	5	6	6	**5.7**	5	4	7	**5.3**
Sensation	Right				Left			
Light Touch								
Upper Extremity Dermatomes	Intact				Intact			
Lower Extremity Dermatomes	Intact				Intact			
Sharp Touch								
Upper Extremity Dermatomes	Intact				Intact			
Lower Extremity Dermatomes	Intact				Intact			
Deep Tendon Reflexes	Right				Left			
Biceps	2+ - Normal				2+ - Normal			
Brachioradials	2+ - Normal				2+ - Normal			
Triceps	1+ - Diminished				1+ - Diminished			
Patella	2+ - Normal				2+ - Normal			
Achilles	2+ - Normal				2+ - Normal			
Neural Dynamics	Right				Left			
Slump	WNL				WNL			
Sciatic	++ Adverse neural dynamics				+ Adverse neural dynamics			
Femoral	NT				NT			
Median	++ Adverse neural dynamics				+ Adverse neural dynamics			
Ulnar	WNL				WNL			
Radial	WNL				WNL			